

SR-001: Lin Chen — Interview Transcript

Interview Transcript

PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2 (Synthetic Interview Data)	Participant ID	SR-001
Persona	Senior Resident	Next Step	Feed into `/summarize-interview` with other SR interviews

SESSION DETAILS

Date	2026-04-02	Duration	40 min
Interviewer	PM Lead	Setting	Face-to-face, conversational
Location	Lin's home, kitchen table (tea served, biscuits)		

PART A — Narrative Transcript

Opening & Warm-Up

Q: Tell me about your daily routine --- what does a typical day look like for you?

I get up at six-thirty, make tea, have breakfast. I used to walk around the block at seven-thirty every morning but I haven't done that for a while now. The weather hasn't been great. I read the news on my iPad, do the crossword. Maria comes on Thursdays — she's my nurse. Sarah comes Wednesdays and Saturdays. The rest of the time I'm here. I watch television. I read. I talk to the grandchildren on Sunday — they video call me on the iPad.

[Probe] You mentioned you haven't been walking. How long has it been?

A few weeks maybe. The footpath near the corner is uneven and I don't trust my balance the way I used to. But I'm fine inside. I hold onto the kitchen bench and the hallway wall. It's just being careful. Everyone makes a fuss about nothing.

[Emotional note] Dismissive tone, but Lin unconsciously gripped the table edge as she spoke. Demonstrates the gap between what she says and what she does.

Block 1: Falls (adapted for SR)

Q: Can you tell me about the fall you had a few months ago?

I tripped on the rug. Two in the morning, going to the bathroom. Caught my foot and went down on my hip. Stupid thing. I should have thrown that rug out years ago. I couldn't get up — the phone was in the bedroom and I couldn't reach it. So I waited until Sarah called in the morning.

[Probe] What was going through your mind during those hours?

I was angry. Mostly at myself. And cold — the hallway floor is tiles. I wasn't scared exactly. More... frustrated. I kept thinking, 'I can handle this. I just need to wait.' I wasn't going to call an ambulance for a bruised hip. By the time Sarah called and I couldn't answer, she came over. Made a big fuss. Bought me that pendant thing.

Q: The pendant alarm --- I understand you stopped wearing it?

I wore it for two days. Then I caught myself in the bathroom mirror wearing this plastic thing around my neck and I looked like a hospital patient. In my own home. In my own bathroom. I took it off and put it in the drawer.

Sarah was upset but I told her I'd wear it at night. I don't wear it at night. Here's the thing nobody understands: everything I do is proof that I can still live here. Every pill I take on time, every morning I make my own tea, every walk around the block. The moment I start wearing emergency buttons and setting alarms for my medication, I've admitted something I'm not ready to admit.

[Emotional note] Lin said this with absolute clarity and conviction. This is not confusion --- this is a calculated strategy for maintaining independence.

Block 2: Medications (adapted for SR)

Q: How do you manage your medications?

Sarah fills the pill organiser on Sundays. I take the morning ones with breakfast, the afternoon ones after lunch, the evening ones after dinner. It's straightforward.

[Probe] Has there ever been a time you weren't sure if you'd taken a dose?

Sometimes. If I get distracted — the phone rings or something's on the television — I can't always remember if I took them or not. If I'm not sure, I usually skip it. Better safe than sorry. Once or twice I've taken them again just in case. It's not ideal but it's manageable.

Q: Has Sarah ever found the organiser didn't look right?

[Pause] She checks it every time she comes. She counts things. She notices if the wrong day is empty. *[Longer pause]* I sometimes... adjust it. If I realise I've missed a dose before she visits, I move the pills around so it looks right. It's not lying — it's managing. If I tell her every little thing, she'll decide I can't manage. And then it starts. The conversations about moving, about more help, about not living alone. I'm not ready for that.

[Emotional note] Lin maintained eye contact throughout. This was not a confession --- it was an explanation. She sees this as rational self-preservation.

I move my walking shoes too. Maria checks things like that. If the shoes are in the same spot every week, she'll know I haven't been out. So I move them to different places by the door. It's not much, but it buys me time.

Block 3: Communication (adapted for SR)

Q: When decisions are made about your care, how are you involved?

I'm not. Sarah talks to Maria. Maria talks to the doctor. Then Sarah calls me and tells me what they decided. I'm right here. Nobody asks me. I taught mathematics for thirty-five years. I can understand my own blood pressure numbers, thank you. I'd like to know what my care plan says. I'd like to know when my next visit is without calling Sarah. I'd like to be a participant, not a subject.

Block 4: Tools & Trust (adapted for SR)

Q: When Maria visits, what's that experience like?

Maria is wonderful. She comes in, she sits down, she accepts a cup of tea, she asks about the grandchildren. Then she does her nurse things. She's a person first and a nurse second. That matters to me. But once — about a month ago — a different nurse came. Maria was sick. This woman I'd never seen walked up to my door and said she was my nurse for today. I didn't let her in. How am I supposed to know who she is? Anyone could say that. Maria, I know. This person, I don't. I told Sarah the nurse didn't come. I didn't tell her I'd turned someone away.

[Probe] What would have needed to be different for you to let her in?

If Maria had called me the day before and said, 'Lin, I'm not feeling well. Karen is going to come instead of me. She's very nice. She knows you like tea first.' Something human. Not a stranger appearing at my door unannounced.

Q: Some nurses use their phones a lot during visits. What's your experience?

The one I turned away — when she came back the next week with a proper introduction, she spent the whole visit typing on her phone. Didn't look at me. Asked me questions like I was at a drive-through. Tick tick tick on her phone. I'm not a checklist. Maria never does that. She looks at me when I talk.

Block 5: Willingness to Prioritize (adapted for SR)

Q: If there were a way to see your own care information --- when your next visit is, what your care plan says --- would you use that?

Yes. I don't want another gadget. I don't want anything around my neck or on my wrist. But if I could ask my iPad, 'When is Maria coming?' or 'What did the doctor change last time?' — I'd use that. I use the iPad for everything else. Why not this?

Q: What about something that monitors your health passively?

Define 'monitors.' If you mean something that watches me and reports to Sarah — no. Absolutely not. I'm not a prisoner. But if it meant that Maria had better information when she visits — she already checks my blood pressure, she might as well have a history of it — I could live with that. As long as it's for her work, not Sarah's anxiety.

Q: You mentioned the GP appointment you cancelled. Can you tell me about that?

I cancelled the doctor because I didn't want them to find something that would give Sarah ammunition. If the doctor says my blood pressure is high or my balance isn't what it was, Sarah finds out, and then it starts. I'm not afraid of dying. I'm afraid of losing my home. Everything I do is proof I can still live here. The moment I can't prove that, they'll move me.

PART B — Filled Note Template

Participant ID	SR-001
Persona	SR
Date	2026-04-02
Duration (min)	40
Interviewer(s)	PM Lead
KEY JOBS	Maintain independence and remain in own home. Prove to Sarah and the system that she can self-manage. Preserve dignity and agency. Manage health enough to avoid the 'care home conversation.'
CURRENT SOLUTION	Pill organiser (filled by Sarah, gamed by Lin). Morning routine as self-proof of capability. Tea with Maria as the human anchor of care. iPad for grandchildren/news. Strategic management of information flow to Sarah. "I move the pills around so it looks right. It's not lying — it's managing. If I tell them every little thing, they'll decide I can't manage." "Nobody asks me. I taught mathematics for thirty-five years. I can understand my own blood pressure numbers." "I'm not afraid of dying. I'm afraid of losing my home." "As long as it's for Maria's work, not Sarah's anxiety."
WILLINGNESS	Would use iPad-based access to own care info. Would accept passive monitoring IF data serves the nurse, not the daughter. Will not wear anything. Will not use anything that feels like surveillance. Will actively sabotage any system that threatens her independence.
EMOTIONAL INTENSITY	Absolute clarity when explaining pill gaming — rational, not confused. Conviction about independence: 'everything I do is proof.' Dismissive about falls but gripped table edge unconsciously. Controlled anger about being excluded from care decisions.

ESCALATION BEHAVIOR	Lin DE-ESCALATES. She conceals symptoms to prevent escalation by others. Hidden dizzy spells (3 months). Skipped GP to avoid diagnosis. Games pill organiser to prevent Sarah's concern. This is REVERSE escalation — the patient actively suppresses signals.
SURPRISE FINDING	DATA GAMING: Lin deliberately manipulates pill organiser and walking shoes to fake adherence + activity. This is strategic, not confused. Also: skipped GP to avoid giving Sarah 'ammunition.' Also: receptive to iPad-based care info access — wants to be a participant, not a subject. Also: the replacement nurse refusal was about trust and introduction, not capability.
TOOL MENTIONS	Pill organiser (gamed). Pendant alarm (drawer). iPad (video calls, news, crossword — receptive to care info). Walking shoes (repositioned).
REFERRAL	Maria Santos ('she's the only one I trust'). Her neighbour Pat ('she checks on me too, informally').
FOLLOW-UP ACTIONS	[] Design for signal suppression: absence of data as a risk signal [] Patient-facing care info access via familiar device (iPad) [] Trust-based carer matching + advance introduction protocol [] Distinguish monitoring-for-nurse vs monitoring-for-family in data access